

CLIENT INTELLIGENCE BRIEF

Complete Public Comment Analysis

CY 2027 Physician Fee Schedule - RPM/RTM Third-Party Clinical Staffing

Prepared for: CareTrack

Prepared by: True North Strategies

Docket: CMS-2026-2377; document CMS-2026-2377-0002

Snapshot: August 3, 2026, approximately 8:13 PM EDT

Status: Interim public record; comment period closes September 14, 2026

1,595 comment records	100% detail records retrieved	113 / 113 attachments extracted
99 RPM/RTM comments	82 employee-only policy comments	79 oppose or seek narrowing

Executive answer

The earlier top-100 list was not the first 100 comments. It was a ranking based on the 456 detailed records available at that time. This replacement analysis retrieves and codes all 1,595 public comment detail records and all 113 supplemental attachments. It therefore replaces the earlier sample estimates with exact snapshot counts.

Bottom line The public record strongly favors replacing the direct-employee test with oversight and integration standards. Among 82 comments that directly address the employment restriction, 79 (96.3%) ask CMS to withdraw, narrow, or replace it; three support or seek clarification of the restriction. This is an administrative-record count, not a public-opinion poll.

What matters for CareTrack

- CareTrack's proposed integrity-plus-access position is well aligned with the dominant substantive argument in the record: employment status is a weak proxy for clinical integration, while supervision, documentation, escalation, and practitioner accountability are directly measurable.
- Volume alone is not enough. Two coordinated RPM/RTM clusters account for 25 policy-specific records. After near-duplicate adjustment, the 82 policy comments represent approximately 59 distinct language units.
- The strongest opposition evidence comes from small practices, rural providers, rehabilitation clinicians, device and monitoring companies, and physicians describing discontinuation risk, staffing infeasibility, and early-intervention benefits.
- The strongest pro-restriction comments focus on billing-driven vendors, fragmented workflows, data that never changes a care plan, and lack of accountability. CareTrack should answer those points directly rather than argue for unrestricted outsourcing.
- Seven Georgia RPM/RTM comments are already in the record and all seven oppose or seek narrowing of the employee-only policy. They offer an immediate base for Georgia-specific outreach.

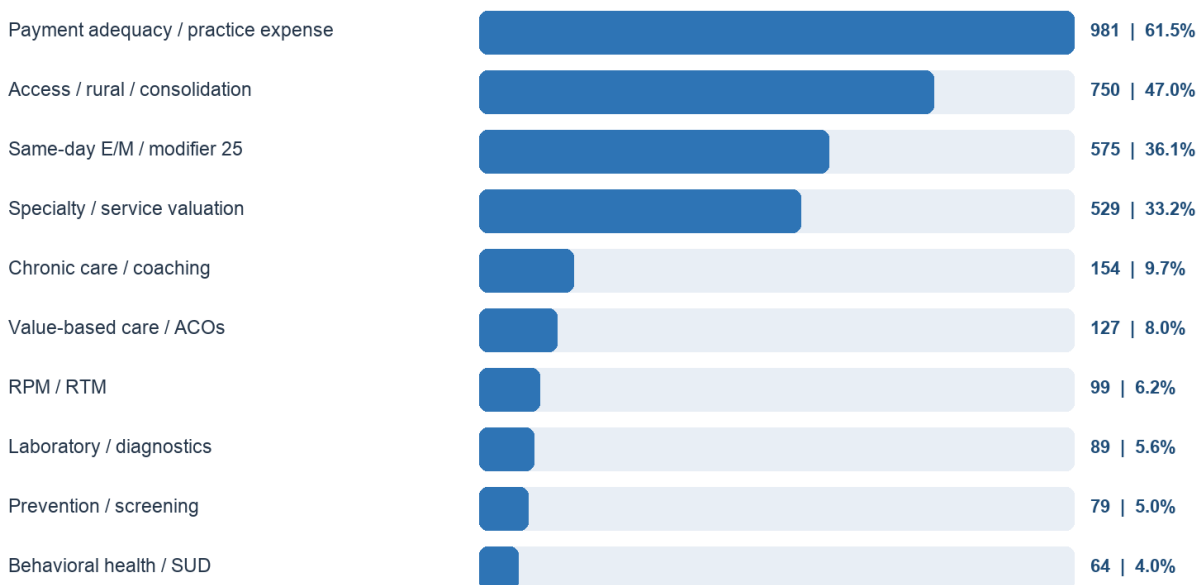
Corpus at a glance

Measure	Exact snapshot result	Interpretation
Public comment records	1,595	All detailed records retrieved
Combined words coded	549,154	Inline bodies plus attachment text/OCR
Named people/organizations	1,202	Unique public names after exact normalization
Anonymous/withheld records	345	Identity unavailable or explicitly anonymous
Comments with attachments	102	102 submissions; 113 logical files
Near-duplicate clusters	75	Heuristic; threshold favors precision
Records in those clusters	348	Campaign volume remains meaningful
Unique-language units	1,322	Approximate after within-cluster deduplication

Themes across the complete record

Theme coding is multi-label: a comment can address several policy issues, so percentages sum to more than 100%. Counts below are exact outputs of the documented keyword rules applied to the full accessible text corpus; they are not extrapolations from a sample.

Themes across all 1,595 comments and attachments



Multi-label coding; totals exceed 100%. Includes machine-readable attachment text and OCR.

Theme	Comments	Share
Payment adequacy, conversion factor & practice expense	981	61.5%
Access, rural burden & consolidation	750	47.0%
Same-day E/M, modifier 25 & global procedures	575	36.1%
Specialty and service valuation	529	33.2%
Chronic care, coaching & navigation	154	9.7%
Value-based care, ACOs & MSSP	127	8.0%
Remote physiologic/therapeutic monitoring	99	6.2%
Laboratory, diagnostics & interoperability	89	5.6%
Prevention, vaccines & screening	79	5.0%
Behavioral health and substance use	64	4.0%
AI, software and human oversight	44	2.8%
Telehealth and virtual access	29	1.8%
Drug payment, pharmacy & 340B	25	1.6%
Wound care & skin substitutes	11	0.7%
Medicare eligibility & immigration	1	0.1%

Theme interpretation

- Payment adequacy is the dominant concern. Commenters connect conversion-factor and practice-expense policy to labor, supply, technology, insurance, and compliance costs that have risen faster than Medicare payment.

- Access is the principal consequence frame. The record repeatedly predicts fewer Medicare appointments, longer travel, delayed diagnosis, practice sales, and migration to higher-cost hospital settings.
- Same-day E/M and modifier 25 policy is the largest discrete campaign. Dermatology, otolaryngology, orthopedics, and other office-based specialties argue that CMS would penalize efficient, whole-patient care.
- RPM/RTM is smaller in docket volume but unusually focused: its comments converge on a specific legal and operational choice-employment status versus measurable integration and accountability standards.

Campaign and duplication effects

The model identified 75 exact or high-similarity clusters containing 348 records. Treating each cluster as one language unit produces approximately 1,322 unique-language units across the docket. The method deliberately excludes short attachment cover notes from duplicate treatment because their underlying files may differ.

Cluster size	Representative subject	Interpretive note
39	I am writing to provide comments on the proposed rule titled "Medicare and Medicaid Programs; CY 2027 Payment ...	Coordinated language; count as mobilization and one principal argument
21	Absolutely yes! Nurse coaches are what's been missing and what is needed to address the growing problems we ar...	Coordinated language; count as mobilization and one principal argument
17	I appreciate the opportunity to comment on the Centers for Medicare & Medicaid Services CY 2027 Medicare Physi...	Coordinated language; count as mobilization and one principal argument
15	If you'd like to copy and paste : Re: Medicare and Medicaid Programs: Calendar Year 2027 Payment Policies unde...	Coordinated language; count as mobilization and one principal argument
15	Dear Administrator, I am writing to urge CMS to address the well-documented sex-based disparities in surgical ...	Coordinated language; count as mobilization and one principal argument
15	I work at Ada West Dermatology in Idaho and am submitting this comment as an individual healthcare employee re...	Coordinated language; count as mobilization and one principal argument
12	I am an orthopedic surgeon practicing at OSMS in Appleton, WI. I am writing to strongly oppose the proposed re...	Coordinated language; count as mobilization and one principal argument
11	I am writing to express concern about the proposed CMAA billing codes, which would allow hospitals to bill for...	Coordinated language; count as mobilization and one principal argument

Client implication Use both numbers in advocacy: 79 opposition records demonstrate salience and mobilization; approximately 59 distinct policy-language units demonstrate that the argument is not solely one copied campaign.

RPM/RTM employee-only proposal: complete record

Measure	Count	Share of 82
Oppose, narrow, or replace employee-only test	79	96.3%
Support or clarify employee-only approach	3	3.7%
Heuristic unique-language units	59	72.0% of records
Records in two coordinated clusters	25	30.5% of records

Arguments made against the restriction

- Access disruption - 77 comments (93.9%). Practices say they would discontinue or shrink monitoring rather than hire a dedicated internal team.
- Workforce shortages - 73 comments (89.0%). Commenters describe specialized, after-hours, and continuous monitoring labor as difficult to recruit and inefficient to duplicate practice by practice.
- Oversight/accountability guardrails - 60 comments (73.2%). Opponents repeatedly volunteer supervision, documentation, escalation, licensing, and audit conditions.
- Program integrity/OIG - 58 comments (70.7%). Most opposition does not deny the integrity problem; it argues the employment test is poorly calibrated to it.
- Alternative to employment test - 52 comments (63.4%). Integration- or performance-based conditions are the dominant replacement framework.
- Continuity/early intervention - 53 comments (64.6%). Comments cite detection of deterioration, adherence problems, device alerts, and avoidable emergency use between visits.
- Clinical integration - 44 comments (53.7%). The treating practice's control of the care plan, protocols, record, and escalation pathway is offered as the quality test.
- Rural/underserved impact - 32 comments (39.0%). Rural workforce scarcity and travel burden are repeatedly used to show disproportionate impact.

The counterargument CareTrack must answer

Three comments support or seek clarification of an employee-centered approach. Their concerns are substantively important even though they are few:

- Some third-party programs are designed around reimbursement rather than measurable clinical action.
- Monitoring data and generated documentation may sit outside the treating team's active care plan.
- A vendor relationship can become a loose association in which the billing practice cannot identify who performed the work or what changed for the patient.
- Technology licensing should remain permissible when the practice's own employees perform the clinical work; CMS should distinguish tools from outsourced clinical services.

Strategic response Do not defend all third-party models. Define the behaviors CareTrack rejects, then ask CMS to condition payment on integration, accountability, and documented clinical action-standards that apply equally to employees and contractors.

Stakeholder landscape

Named policy commenters opposing or narrowing the restriction include independent practices, monitoring and device companies, rehabilitation providers, and rural institutions. Illustrative

organizations include CSRA Renal Services, rtNOW, Xandar Kardian, PhyxUp Health, Precision Pain Care, Esvyda, InVite Fitness, Upstream Rehabilitation, Code Pause, Limestone Medical Center, Massive Bio, SurgicalRx, FirstVitals/ProteusDx, Illinois Rural Community Care Organization, and Mallard Medical.

The organization field is incomplete and self-reported; several physician and executive comments identify affiliations only in the text. This list is a coalition-development lead set, not an audited registry of legal entities.

Georgia record and advocacy value

Seven Georgia RPM/RTM submissions directly oppose or seek narrowing of the employee-only proposal:

- Krisi Probert - Health Care Industry - PI015; CMS-2026-2377-1265.
- Matthew Johnson - Individual; CMS-2026-2377-1293.
- Jennifer Cazeaux - Individual; CMS-2026-2377-1306.
- Mason Williams - Private Industry - Health Care; CMS-2026-2377-1349.
- Stephen Weyandt - Health Care Professional/Association - Physical Therapist; CMS-2026-2377-1384.
- Eric Steed - Health Care Professional/Association - Physical Therapist; CMS-2026-2377-1388.
- Matthew Foley - Health Care Professional/Association - Physical Therapist; CMS-2026-2377-1619.

Mason Williams identifies himself in the comment as owner of Mallard Medical, a Georgia remote-monitoring provider. The remaining Georgia record includes rehabilitation professionals and an Upstream Rehabilitation formal letter. These commenters can validate access and staffing arguments independently of CareTrack if willing to engage publicly.

Recommended CareTrack position

Primary ask Withdraw the direct-employee condition and preserve payment for contracted clinical staff when the billing practitioner retains documented clinical control, supervision, medical-necessity responsibility, and claim accountability.

Offer CMS a complete guardrail package rather than a general promise of compliance:

1. A written agreement identifies the billing practice, contracted entity, responsible practitioner, clinical staff, services, and escalation duties.
2. The treating practitioner establishes medical necessity, orders monitoring, approves enrollment, sets the plan of care, and periodically reaffirms continued need.
3. Third parties may not independently solicit, diagnose, enroll, or determine that a Medicare claim should be submitted.
4. Contracted clinical staff are appropriately licensed, assigned to a defined practice panel, trained on practice-approved protocols, and identified in the record.
5. Monitoring data, communication, time, alerts, and clinical responses are integrated into the practice's designated medical record-not maintained only in a parallel vendor system.

6. The practice controls escalation thresholds and documents practitioner review and action on clinically significant findings.
7. Claims or supporting records disclose contracted performance; the billing practitioner attests to supervision and remains liable for medical necessity and billing accuracy.
8. CMS may audit enrollment, communication, time, intervention, and outcome records and terminate non-integrated or solicitation-driven arrangements.

Evidence CareTrack should supply

- Number of Georgia Medicare beneficiaries monitored, by county and congressional district; identify rural and health-professional-shortage areas.
- Number and type of Georgia practices served, including practice size, specialty, staffing model, and whether they could economically internalize monitoring.
- Actual clinical workflow: enrollment, consent, data review, escalation, practitioner communication, record integration, and pre-claim review.
- Quantified interventions and outcomes: alerts escalated, medication changes, urgent visits initiated, emergency visits avoided, admissions/readmissions, patient retention, and response time-with methodology and denominators.
- A transition-impact model comparing contracted staffing, in-house staffing, program discontinuation, and a one-year delay.
- Compliance controls that distinguish CareTrack from the solicitation and fragmented-care models described by OIG and supportive commenters.

Recommended next 14 days

9. Finalize a two-page regulatory alternative built around the eight guardrails above, including proposed conditions-of-payment language.
10. Recruit three to five independent Georgia practices for individualized comments using real staffing and patient examples; avoid copy-and-paste letters with visible placeholders.
11. Contact the seven Georgia RPM/RTM commenters and the named national organizations to test interest in aligned but independently authored filings.
12. Prepare a one-page rebuttal to the three supportive comments: acknowledge billing-driven vendors and show how CareTrack's model creates documented clinical action.
13. Develop a small-practice cost model and rural patient-impact map before congressional district meetings.
14. Request delay to January 1, 2028 as a fallback, tied to CMS guidance, registration, reporting, and transition standards-not as an unconditioned reprieve.

Complete submitter census

The companion submitter CSV ranks all 1,202 named people and organizations observed across the 1,595-record census. The maximum observed count for one public name is five; four names appear four

times; three names or organizations appear three times. Most submitters appear once. Counts should not be interpreted as influence or organizational endorsement without entity verification.

Rank	Submitter	Type	Records
1	Amy Holton	Individual	5
2	David Rocha	Individual	4
3	Richard Hope	Individual	4
4	Scott Goldberg	Individual	4
5	Ada West Dermatology	Organization	3
6	Elnara Muradova	Individual	3
7	Stewart Bernard	Individual	3
8	ABC Coding Solutions, Inc.	Organization	2
9	Amar Patel	Individual	2
10	Amy Schulz	Individual	2
11	Angela Ellerman	Individual	2
12	Billy Rutter	Individual	2
13	Blue Valley Dermatology	Organization	2
14	Bonnie Pewterbaugh	Individual	2
15	Cathy Gilges	Individual	2
16	Charles Phipps	Individual	2
17	Chelsea Duggan	Individual	2
18	Cindy Wassef	Individual	2
19	David Doud	Individual	2
20	Ismar Dizdarevic	Individual	2
21	Jennifer Haase	Individual	2
22	Kevin VIEL	Individual	2
23	Kim Dakin	Individual	2
24	Kristin Fields	Individual	2
25	Laleh Bedocs	Individual	2

Method, auditability, and limitations

- Snapshot scope: 1,595 public comment records posted to CMS-2026-2377-0002 as of approximately 8:13 PM EDT on August 3, 2026. The docket remains open.
- Coverage: every public detail record was retrieved. All 113 logical attachments were downloaded; 108 yielded machine-readable text and five image-only files were OCR-processed. Theme coding uses the combined inline and supplemental text.
- Identity: names, organizations, categories, and states are public, agency-configurable, self-reported, and sometimes withheld. Entity names were grouped by exact displayed text but not legally verified or merged across spelling variants.
- Themes: rules are deterministic, multi-label keyword classifications. They improve consistency and auditability but may still produce false positives or false negatives; close legal or policy judgments were manually reviewed for the 82 employee-only comments.
- Duplication: exact and five-word-shingle near-duplicate clustering uses a conservative similarity threshold. The resulting unique-language count is heuristic, not a legal determination of originality.

- Attachments: OCR can misread handwriting, low-resolution scans, and unusual layouts. Five scanned files were reviewed through OCR; their primary inline comments and metadata remain available even if some characters were imperfect.
- Administrative-record meaning: comment volume demonstrates salience and mobilization, not representativeness. CMS may give greater weight to evidence, legal analysis, operational specificity, and workable alternatives than to raw counts.

Companion data files

- Complete named submitter census - all 1,202 public names and organizations with comment IDs.
- All-comments coding table - one row per comment with theme flags, dates, identity fields, word counts, and attachment counts.
- RPM/RTM policy map - all 99 related comments, with employee-only stance and subtheme flags.
- Near-duplicate cluster table - cluster membership and representative excerpts for audit and campaign analysis.

Sources

[Regulations.gov live comments](#) - Primary public record and attachment source.

[Regulations.gov v4 API documentation](#) - Field definitions, detail endpoints, rate limits, and data limitations.

[CMS CY 2027 PFS proposed-rule materials](#) - Official proposed rule and supporting files.

Client context: CareTrack-RPM-Advocacy-Proposal.md, provided August 3, 2026.

Comprehensive evidence appendix

Full-corpus exhibits, stakeholder intelligence, RPM/RTM policy analysis, and auditable appendices

How to use this appendix The first nine pages are the executive client brief. The pages that follow add the underlying distributions, intersections, policy record, stakeholder lists, top-100 ranking, coding definitions, and audit guidance. Counts are a snapshot of the open docket, not a forecast of the final record.

Scope and evidence hierarchy

- Census layer: 1,595 public detail records were retrieved, rather than a convenience sample or the first 100 results.
- Supplement layer: all 113 logical attachments were included; 108 were machine-readable and five image-only files were OCR-processed.
- Coding layer: every record received deterministic, multi-label theme flags. These are discovery aids and counts, not legal adjudications.
- Manual layer: all 82 comments directly addressing the employee-only RPM/RTM staffing restriction received manual stance review.
- Client layer: recommendations translate the record into CareTrack positioning; they are strategic judgments and are labeled accordingly.

Expanded headline findings

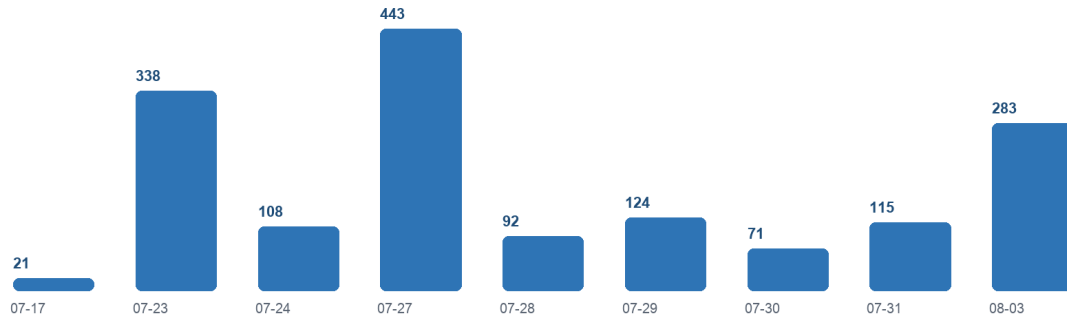
- Economic pressure and access are the docket's central narrative. Payment adequacy appears in 981 comments, access/rural burden/consolidation in 750, and both appear together in 605 comments (37.9% of the entire corpus).
- Same-day E/M and modifier 25 is a large, coordinated policy campaign: 575 comments mention the issue, and 407 combine it with an access argument.
- The docket contains meaningful mobilization effects: 348 records fall within 75 exact or high-similarity clusters. A conservative adjustment yields approximately 1,322 unique-language units.
- RPM/RTM is smaller in raw volume (99 comments) but unusually concentrated around an implementable policy choice. Among the 82 direct employee-only comments, 79 oppose, narrow, or replace the condition and three support or clarify it.
- Metadata is incomplete. State is reported for 941 records (59.0%); organization fields capture only entities entered into the public organization field, not every affiliation mentioned inside comment text.

Corpus composition and timing

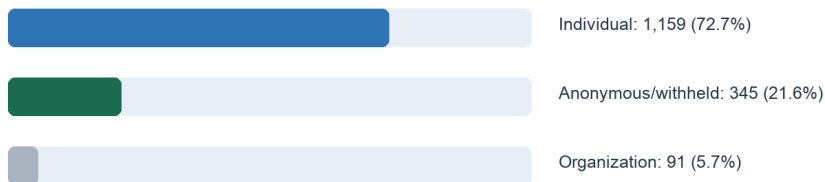
Who submitted, when they submitted, and what the public metadata can support

Corpus composition and submission activity

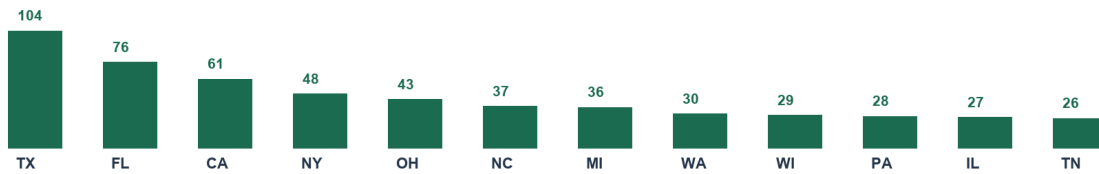
Daily public comments



Public identity fields



Top reported states (654 records did not report a state)



Timing

The snapshot contains comments posted on 9 dates between 2026-07-17 and 2026-08-03. The peak was 2026-07-27 with 443 comments (27.8% of the census). Concentrated filing days are consistent with organized outreach, but timing alone does not prove common authorship.

Posted date	Comments	Cumulative	Share
2026-07-17	21	21	1.3%
2026-07-23	338	359	21.2%
2026-07-24	108	467	6.8%
2026-07-27	443	910	27.8%
2026-07-28	92	1002	5.8%
2026-07-29	124	1126	7.8%
2026-07-30	71	1197	4.5%
2026-07-31	115	1312	7.2%
2026-08-03	283	1595	17.7%

Stakeholder composition

Public identity fields, categories, and organizational participation

Submitter identity

Public identity type	Records	Share
Individual	1159	72.7%
Anonymous/withheld	345	21.6%
Organization	91	5.7%

The 1,159 individual records dominate the docket. Ninety-one records use an organization as the public submitter, representing 86 distinct public organization-field values. Another 345 records are anonymous or withheld. The public identity fields do not reliably capture every employer, association, vendor relationship, or client affiliation described inside narrative text.

Leading self-selected categories

Category	Records	Share
Health Care Professional/Association - Physician	472	29.6%
Unspecified	374	23.4%
Physician - HC005	167	10.5%
Health Care Industry - PI015	90	5.6%
Individual	57	3.6%
Health Care Professional/Association - Nurse	46	2.9%
Government - Federal	33	2.1%
Academic - OT005	24	1.5%
Health Care Professional/Association - Other Health Care Professional	23	1.4%
Health Care Professional/Association - Physical Therapist	22	1.4%
Health Care Provider/Association - Other	22	1.4%
Health Care Professional/Association - Speech-Language Therapist	19	1.2%
Health Care Professional/Association - Physician Assistant	19	1.2%
Other - OT001	15	0.9%
Speech-Language Therapist - HC055	14	0.9%
Other Health Care Professional - HC075	14	0.9%
Nurse - HC065	14	0.9%
Federal Government - G0005	14	0.9%
Private Industry - Health Care	14	0.9%
Congressional	13	0.8%

Interpretation caution Category labels are agency-configurable and self-selected. Similar constituencies appear under legacy and newer labels (for example, two physician categories), so category rows should not be treated as mutually exclusive professional taxonomies.

Geographic record

Reported-state distribution and limitations

A state or jurisdiction is reported for 941 records (59.0%). The data contain 53 distinct reported state/jurisdiction codes; 654 records are unspecified. Consequently, state rankings describe known metadata rather than the actual geographic composition of every submitter.

State	Records	Share of all records
Unspecified	654	41.0%
TX	104	6.5%
FL	76	4.8%
CA	61	3.8%
NY	48	3.0%
OH	43	2.7%
NC	37	2.3%
MI	36	2.3%
WA	30	1.9%
WI	29	1.8%
PA	28	1.8%
IL	27	1.7%
TN	26	1.6%
GA	26	1.6%
ID	24	1.5%
VA	22	1.4%
MN	21	1.3%
SC	19	1.2%
MO	19	1.2%
IN	18	1.1%
NJ	18	1.1%
AZ	18	1.1%
CO	17	1.1%
OK	15	0.9%
OR	15	0.9%
KS	14	0.9%
KY	14	0.9%
MD	14	0.9%
NV	12	0.8%
NH	10	0.6%

Georgia

Georgia appears on 26 comments overall and seven direct RPM/RTM employee-only comments. All seven of the latter oppose, narrow, or replace the proposed restriction. This creates a credible state-specific advocacy base, but it should be supplemented with new, individualized evidence from practices that can quantify patient and staffing effects.

Comment depth and discourse structure

Length, attachment use, theme intersections, and campaign effects

Length and attachment profile

The combined inline-plus-attachment corpus contains 549,154 words. Median length is 178 words; the 25th and 75th percentiles are 81 and 388. The longest submission contains 27,518 words. Long attached letters can therefore contribute far more substantive text than short portal entries.

Combined length	Comments	Share
Under 50 words	229	14.4%
50-199 words	635	39.8%
200-499 words	457	28.7%
500-999 words	216	13.5%
1,000-2,499 words	36	2.3%
2,500+ words	22	1.4%

Strongest theme intersections

Theme 1	Theme 2	Records	Corpus share
Payment adequacy, conversion factor & practice expense	Access, rural burden & consolidation	605	37.9%
Access, rural burden & consolidation	Same-day E/M, modifier 25 & global procedures	407	25.5%
Payment adequacy, conversion factor & practice expense	Same-day E/M, modifier 25 & global procedures	406	25.5%
Payment adequacy, conversion factor & practice expense	Specialty and service valuation	393	24.6%
Access, rural burden & consolidation	Specialty and service valuation	348	21.8%
Same-day E/M, modifier 25 & global procedures	Specialty and service valuation	253	15.9%
Payment adequacy, conversion factor & practice expense	Chronic care, coaching & navigation	112	7.0%
Payment adequacy, conversion factor & practice expense	Value-based care, ACOs & MSSP	98	6.1%
Payment adequacy, conversion factor & practice expense	Remote physiologic/therapeutic monitoring	89	5.6%
Payment adequacy, conversion factor & practice expense	Laboratory, diagnostics & interoperability	74	4.6%
Access, rural burden & consolidation	Remote physiologic/therapeutic monitoring	69	4.3%
Payment adequacy, conversion factor & practice expense	Prevention, vaccines & screening	62	3.9%

Client implication The strongest persuasive bridge is not RPM/RTM technology alone. It is the link between staffing flexibility, accountable clinical integration, patient access, and the broader economics of independent practice.

Theme-by-theme interpretation

All coded themes across the complete record

Payment adequacy, conversion factor & practice expense

981 comments (61.5%). The dominant economic frame. Commenters connect nominal payment changes, practice expense methodology, and efficiency assumptions to viability and service availability.

Access, rural burden & consolidation

750 comments (47.0%). The principal consequence frame. Commenters predict longer travel, delayed care, reduced appointments, practice sales, and migration to higher-cost settings.

Same-day E/M, modifier 25 & global procedures

575 comments (36.1%). A major organized campaign, especially among office-based specialties. The recurring claim is that payment policy would discourage efficient diagnosis and treatment in one encounter.

Specialty and service valuation

529 comments (33.2%). A broad collection of specialty-specific code and valuation concerns. It should be disaggregated before making specialty-level conclusions.

Chronic care, coaching & navigation

154 comments (9.7%). Support for reimbursed coaching and navigation appears alongside questions about qualifications, supervision, care-plan integration, and duplication with existing services.

Value-based care, ACOs & MSSP

127 comments (8.0%). Comments address benchmarks, shared savings, quality programs, and whether fee-schedule changes reinforce or undermine value-based participation.

Remote physiologic/therapeutic monitoring

99 comments (6.2%). A focused operational policy record covering staffing, integration, device supply, codes, oversight, and program integrity. This is CareTrack's core issue.

Laboratory, diagnostics & interoperability

89 comments (5.6%). Concerns include duplicate testing, data exchange, diagnostic valuation, and specialty laboratory policy.

Prevention, vaccines & screening

79 comments (5.0%). Comments support or critique payment and access for immunization, screening, wellness, and prevention services.

Behavioral health and substance use

64 comments (4.0%). Comments address access, workforce, opioid/substance-use treatment, and payment for behavioral health services.

AI, software and human oversight

44 comments (2.8%). Comments debate reimbursement for software-mediated clinical work and insist on transparency, safety, accountability, and appropriate human oversight.

Telehealth and virtual access

29 comments (1.8%). A smaller theme in this snapshot, centered on maintaining virtual access and appropriate treatment of rural clinics and qualified sites.

Drug payment, pharmacy & 340B

25 comments (1.6%). A discrete set of pharmacy, drug-payment, rebate, and 340B issues; it is not central to CareTrack positioning.

Wound care & skin substitutes

11 comments (0.7%). A small but specialized cluster concerning wound management, skin substitutes, and debridement payment.

Medicare eligibility & immigration

1 comments (0.1%). One record triggered this coding theme; no broad docket-wide conclusion should be drawn.

Campaigns, duplication, and independence

How repeated language changes interpretation of volume

The conservative duplicate analysis identifies 75 exact or high-similarity clusters containing 348 records. Collapsing every cluster to one language unit produces approximately 1,322 unique-language units. This is a sensitivity measure, not a claim about authorship, coordination, or legal originality.

Cluster	Records	Representative subject (truncated)
1	39	I am writing to provide comments on the proposed rule titled "Medicare and Medicaid Programs; CY 2027 Payment Policies under the Physician Fee Sc...
2	21	Absolutely yes! Nurse coaches are what's been missing and what is needed to address the growing problems we are facing in healthcare. They can be...
3	17	I appreciate the opportunity to comment on the Centers for Medicare & Medicaid Services CY 2027 Medicare Physician Fee Schedule proposed rule. As...
4	15	If you'd like to copy and paste : Re: Medicare and Medicaid Programs: Calendar Year 2027 Payment Policies under the Physician Fee Schedule and Ot...
5	15	Dear Administrator, I am writing to urge CMS to address the well-documented sex-based disparities in surgical reimbursement rates. Peer-reviewed ...
6	15	I work at Ada West Dermatology in Idaho and am submitting this comment as an individual healthcare employee regarding the proposed Medicare polic...
7	12	I am an orthopedic surgeon practicing at OSMS in Appleton, WI. I am writing to strongly oppose the proposed reimbursement reductions for total hi...
8	11	I am writing to express concern about the proposed CMAA billing codes, which would allow hospitals to bill for clinical work performed entirely b...
9	10	I am writing regarding the proposed Medicare policy affecting evaluation and procedures performed during the same visit. Patients should be able ...
10	9	As an independent private practice provider, we provide care at the lowest price for Medicare. Because we have little voice in politics, we are a...
11	9	Please see attached Ananth Thyagarajan, MD Allergy Partners of Springfield 6035 Burke Centre Parkway, Suite 330 Burke, VA 22015 athyagarajan@alle...
12	8	I'm submitting this comment on the CY 2027 Physician Fee Schedule proposed rule (CMS-1848-P), specifically about Community Health Integration (CH...
13	8	Dr. Mehmet Oz Administrator Centers for Medicare & Medicaid Services 7500 Security Boulevard Baltimore, MD 21244 Re: CMS-1832-P - CY 2027 Physi...
14	7	I am in strong support of the Centers for Medicare & Medicaid Services' (CMS) proposal to establish national payment for individual and group hea...
15	6	Regarding code: CMS-1848-P A Policy Statement Opposing Continued Medicare Physician Payment Reductions for CPT 27130 and CPT 27447 Executive Summ...
16	5	To CMS, I am submitting this comment regarding the CY 2027 Medicare Physician Fee Schedule Proposed Rule and the potential impact of the proposed...
17	5	I am writing to voice my opposition to the proposed cuts to the physician Medicare fee schedule. As an independent physician practice we are espe...
18	5	Nurse Coaches can help bridge the gap between what patients are advised to do and what they are realistically able to implement in their daily li...
19	4	Docket: CMS-1848-P (Medicare Program; CY 2027 Payment Policies) Submitted by: Amy Holton, BSN, RN, PHN Title: Healthtech Ethics & Safety Speciali...
20	4	Dear sir, They proposed role of cutting either the officers or the procedure by 50% when a 25 modifier is used will be terrible policy for the pa...

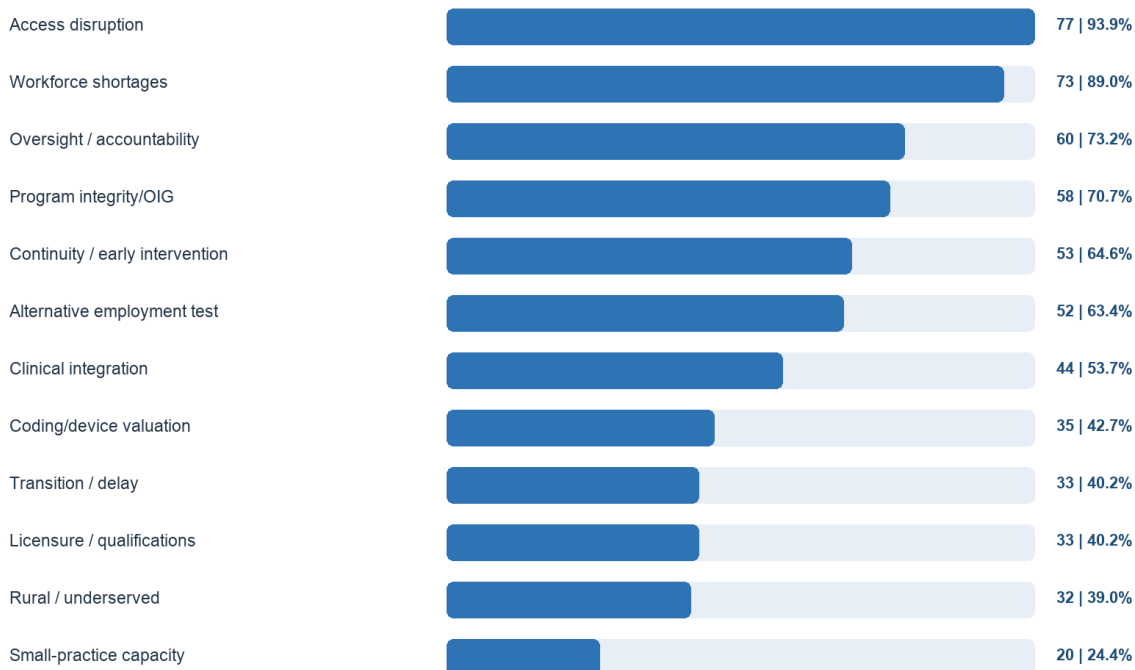
How to report both measures

- Use record count to describe participation and mobilization: each submission is an official docket record.
- Use duplicate-adjusted language units to describe breadth of independently worded argument.
- Do not call template users fake or discount their participation. Repeated language can still show salience, constituency reach, and organized engagement.
- Avoid visible placeholders and identical form letters in future CareTrack outreach; individualized practice facts will carry more evidentiary weight.

RPM/RTM employee-only proposal

Complete policy-specific record and argument architecture

RPM/RTM employee-only policy: argument frequency



Bottom line Among 82 direct comments, 79 (96.3%) oppose, narrow, or replace the employee-only restriction; three (3.7%) support or clarify it. Duplicate adjustment yields approximately 59 unique-language units, including 25 records in two coordinated clusters.

Four-part opposition argument

1. Patient harm: loss or contraction of monitoring could delay detection, escalation, and intervention between visits.
2. Operational infeasibility: small, independent, rural, and specialty practices may not be able to hire and manage a dedicated internal monitoring workforce.
3. Policy mismatch: employment status is presented as an imperfect proxy for clinical integration, supervision, documentation, and accountability.
4. Alternative design: commenters propose measurable guardrails for orders, qualifications, integration, escalation, claim disclosure, and auditability.

Supportive counterargument

The three supportive or clarifying comments emphasize billing-driven vendor models, fragmented monitoring outside the treating team's care plan, uncertain accountability, and technology arrangements that may blur the line between software support and outsourced clinical work. A credible CareTrack position must answer those risks directly rather than treating all third-party models as equivalent.

CareTrack response standard

- Draw a bright operational line between technology licensing and performance of billable clinical work.
- Show that the treating practice - not the vendor - determines medical necessity, enrollment, care plans, escalation thresholds, and claim submission.
- Document how contracted clinicians are licensed, assigned, supervised, integrated into the medical record, and accountable to the billing practitioner.
- Accept claim-level disclosure and auditability so CMS can distinguish integrated models from solicitation-driven or billing-driven arrangements.
- Offer outcome, intervention, continuity, and patient-experience measures that make quality observable without relying on employment status as the proxy.

RPM/RTM policy subthemes and geography

What the 82 direct comments argue and where public state metadata is available

Policy subtheme	Comments	Share of 82
Access disruption	77	93.9%
Workforce shortages	73	89.0%
Oversight/accountability guardrails	60	73.2%
Program integrity/OIG	58	70.7%
Continuity/early intervention	53	64.6%
Alternative to employment test	52	63.4%
Clinical integration	44	53.7%
Coding/device valuation	35	42.7%
Transition/delay request	33	40.2%
Licensure/qualified staff	33	40.2%
Rural/underserved impact	32	39.0%
Small/independent practice capacity	20	24.4%

Reported state distribution

State	Comments	Share of 82
Unspecified	49	59.8%
GA	7	8.5%
PA	4	4.9%
TX	3	3.7%
TN	3	3.7%
KY	2	2.4%
MO	2	2.4%
NC	2	2.4%
CA	1	1.2%
FL	1	1.2%
MS	1	1.2%
UT	1	1.2%
WA	1	1.2%
DE	1	1.2%
AL	1	1.2%
OH	1	1.2%
OR	1	1.2%
NY	1	1.2%

Geographic caution Forty-nine of the 82 direct policy comments (59.8%) do not report a state. Georgia's seven known comments are meaningful for state advocacy, but they do not establish Georgia as the largest actual source of opposition.

CareTrack policy option matrix

A position that addresses both access and program-integrity concerns

Policy option	Access effect	Integrity effect	CareTrack assessment
Finalize direct-employee condition	High disruption risk for practices reliant on contracted clinical teams	Bright-line staffing rule; may simplify attribution	Oppose unless substantially narrowed and delayed
Retain broad current flexibility	Preserves operational access	Does not fully answer concerns about solicitation, fragmented care, or unclear accountability	Insufficient by itself
Integration-and-accountability guardrails	Preserves qualified contracted staffing	Makes orders, supervision, documentation, escalation, disclosure, and audits measurable	Primary recommendation
Hybrid transition / delayed implementation	Reduces abrupt discontinuation	Allows CMS to operationalize reporting and oversight	Fallback paired with guardrails

Recommended eight-part guardrail package

1. A written agreement identifies the billing practitioner, contracted entity, responsible clinicians, services, escalation duties, and record-access obligations.
2. The treating practitioner establishes medical necessity, orders monitoring, approves enrollment, sets the care plan, and performs periodic reassessment.
3. Third parties may not independently solicit, diagnose, enroll, or determine that a Medicare claim should be submitted.
4. Contracted clinical staff must be appropriately licensed, trained, and practice-approved for the assigned monitoring and escalation functions.
5. Monitoring data, communications, time, alerts, interventions, and clinical responses must be integrated into the practice's designated medical record.
6. The practice controls escalation thresholds and documents practitioner review and action on clinically significant findings.
7. Claims or supporting records disclose contracted performance, and the billing practitioner attests to supervision, medical necessity, and accountability.
8. CMS and its auditors can retrieve enrollment, communication, time, intervention, and outcome records and terminate non-integrated or solicitation-driven arrangements.

Georgia and client-specific advocacy

How CareTrack can convert docket support into credible evidence

Comment ID	Public submitter	Category	Stance
CMS-2026-2377-1265	Krisi Probert	Health Care Industry - PI015	Oppose/narrow employment restriction
CMS-2026-2377-1293	Matthew Johnson	Individual	Oppose/narrow employment restriction
CMS-2026-2377-1306	Jennifer Cazeaux	Individual	Oppose/narrow employment restriction
CMS-2026-2377-1349	Mason Williams	Private Industry - Health Care	Oppose/narrow employment restriction
CMS-2026-2377-1384	Stephen Weyandt	Health Care Professional/Association - Physical Therapist	Oppose/narrow employment restriction
CMS-2026-2377-1388	Eric Steed	Health Care Professional/Association - Physical Therapist	Oppose/narrow employment restriction
CMS-2026-2377-1619	Matthew Foley	Health Care Professional/Association - Physical Therapist	Oppose/narrow employment restriction

Evidence CareTrack should assemble

- Georgia beneficiary and practice distribution, segmented by rurality, specialty, and county or congressional district where appropriate.
- A staffing counterfactual: current contracted model versus internal hiring costs, time-to-hire, coverage hours, licensure mix, turnover, and practice-level break-even volume.
- Clinical workflow maps showing who enrolls, reviews data, communicates with patients, escalates findings, documents actions, and remains accountable.
- Outcome and intervention evidence: alerts, practitioner contacts, medication or care-plan changes, avoided urgent events where methodologically supportable, and discontinuation risk.
- Program-integrity controls: no independent solicitation, no vendor-determined billing, claim support, audit logs, licensure checks, escalation rules, and performance monitoring.
- A transition scenario explaining the patient and practice consequences of immediate implementation and why a January 1, 2028 fallback date is operationally reasonable.

Advocacy sequencing

1. Lead with patient access and accountable integration, not vendor economics.
2. Acknowledge the legitimate concern about low-value or fragmented vendor models, then distinguish CareTrack's controls and clinical integration.
3. Recruit three to five Georgia practices for individualized submissions with real staffing, patient, and escalation examples.
4. Coordinate a two-page guardrail alternative and a short transition fallback across practice, clinician, and patient voices without using identical language.
5. Use both numbers accurately: 79 opposition records demonstrate mobilization; approximately 59 unique-language units demonstrate breadth beyond two coordinated clusters.

Top 100 named submitters

Full-census ranking by number of public comment records

Ranking is based on exact displayed public names or organization-field values across all 1,595 records after trimming whitespace. It is not a ranking of influence, comment length, substantive quality, or organizational endorsement. Case and spelling variants were not entity-resolved.

Rank	Submitter	Type	Records	Reported state(s)
1	Amy Holton	Individual	5	MN
2	David Rocha	Individual	4	TX
3	Richard Hope	Individual	4	TX
4	Scott Goldberg	Individual	4	FL
5	Ada West Dermatology	Organization	3	Unspecified
6	Elnara Muradova	Individual	3	MD
7	Stewart Bernard	Individual	3	TN
8	ABC Coding Solutions, Inc.	Organization	2	Unspecified
9	Amar Patel	Individual	2	Unspecified
10	Amy Schulz	Individual	2	NM
11	Angela Ellerman	Individual	2	Unspecified
12	Billy Rutter	Individual	2	OH
13	Blue Valley Dermatology	Organization	2	Unspecified
14	Bonnie Pewterbaugh	Individual	2	FL
15	Cathy Gilges	Individual	2	KS
16	Charles Phipps	Individual	2	TX
17	Chelsea Duggan	Individual	2	FL
18	Cindy Wassef	Individual	2	Unspecified
19	David Doud	Individual	2	MI; Unspecified
20	Ismar Dizdarevic	Individual	2	NJ
21	Jennifer Haase	Individual	2	MD
22	Kevin VIEL	Individual	2	NH
23	Kim Dakin	Individual	2	LA
24	Kristin Fields	Individual	2	NJ
25	Laleh Bedocs	Individual	2	Unspecified
26	Laura Alessio	Individual	2	CA
27	Luke Fraundorf	Individual	2	Unspecified
28	Madison Hill	Individual	2	MD
29	Martha Brown	Individual	2	TN
30	natwarlal ramani	Individual	2	Unspecified
31	Sandi Eivins	Individual	2	CO
32	Sandra Verma	Individual	2	NJ
33	Sreenivasu Odugu	Individual	2	NH
34	St. Luke's Health System	Organization	2	Unspecified
35	Stephanie Savage	Individual	2	NY
36	Veronica Alvarez-Galiana	Individual	2	FL
37	143 Speech Therapy	Organization	1	Unspecified
38	Aanand Geria	Individual	1	Unspecified
39	Aaron Burch	Individual	1	Unspecified
40	Aaron Pace	Individual	1	Unspecified
41	Abby Sipe	Individual	1	Unspecified
42	ABC Coding Solutions	Organization	1	Unspecified
43	Abhyudaya Joshi	Individual	1	NC
44	Abigail Donnelly	Individual	1	IN
45	About Women Ob/Gyn	Organization	1	Unspecified
46	Ada west dermatology	Organization	1	Unspecified
47	Adam Brys	Individual	1	Unspecified
48	Adam Cien	Individual	1	MI
49	Adam Daily	Individual	1	WA
50	Adam Delora	Individual	1	IL
51	Adam Ingraffea	Individual	1	NC
52	Adam Kimple	Individual	1	NC
53	Adam Mariotti	Individual	1	PA
54	Adam McCall	Individual	1	AR
55	Adam Siegel	Individual	1	FL
56	Adam Swigost	Individual	1	TX
57	Adam Wright MD	Individual	1	TX

Rank	Submitter	Type	Records	Reported state(s)
58	Addie Taylor	Individual	1	NC
59	Adriane Owen	Individual	1	FL
60	Adrienne Edgren	Individual	1	MI
61	Adrienne Jordan	Individual	1	Unspecified
62	Ahmed Hawash	Individual	1	Unspecified
63	Ajith Nair	Individual	1	KY
64	Akron Dermatology, Inc.	Organization	1	Unspecified
65	Alaa Kassir	Individual	1	Unspecified
66	Alan Alan	Individual	1	MO
67	Alex Chavez	Individual	1	NC
68	Alexander Marzuka, MD, FAAD	Individual	1	TX
69	Alexandra Fry	Individual	1	Unspecified
70	Alexi Johnson	Individual	1	ID
71	Alfred Fleming	Individual	1	OH
72	Ali Carine	Individual	1	OH
73	Ali Mohammed	Individual	1	DE
74	Alicia Bernard	Individual	1	TN
75	Alicia Graziano	Individual	1	DE
76	Alicia Spence	Individual	1	PA
77	Alina Thompson	Individual	1	CA
78	Alison Auster	Individual	1	CO
79	Alison Black	Individual	1	TX
80	Alison Hanna	Individual	1	WA
81	Alissa Erogbogbo	Individual	1	CA
82	Aliza Mamber	Individual	1	Unspecified
83	Allie Nitsch	Individual	1	TX
84	Allie Pardue	Individual	1	KY
85	Allison Arthur	Individual	1	FL
86	Allon Friedman, MD	Individual	1	IN
87	Alpen Patel	Individual	1	MD
88	Alyse Worley	Individual	1	SC
89	Alyssa Gunther	Individual	1	NC
90	Alyssa Tozzi	Individual	1	Unspecified
91	Amanda Bailey	Individual	1	Unspecified
92	Amanda Dise	Individual	1	WA
93	Amanda Duncanson	Individual	1	ID
94	Amanda Habrat	Individual	1	MN
95	Amanda Rohn	Individual	1	DC
96	Amanda Stephenson	Individual	1	CA
97	Amanda Vignaroli	Individual	1	WY
98	Amanda Zueck	Individual	1	Unspecified
99	Amelia Sutton	Individual	1	NC
100	Amir Aboutalebi	Individual	1	TX

Organization submitter census

All 86 distinct public organization-field values

Rank	Organization	Records	State(s)	Category/categories
1	Ada West Dermatology	3	Unspecified	Health Care Professional/Association - Other Technician; Health Plan or Association; Unspecified
2	ABC Coding Solutions, Inc.	2	Unspecified	Health Care Industry - PI015
3	Blue Valley Dermatology	2	Unspecified	Unspecified
4	St. Luke's Health System	2	Unspecified	Health Care Industry - PI015; Unspecified
5	143 Speech Therapy	1	Unspecified	Speech-Language Therapist - HC055
6	ABC Coding Solutions	1	Unspecified	Government - Federal
7	About Women Ob/Gyn	1	Unspecified	Health Care Provider/Association - Other
8	Ada west dermatology	1	Unspecified	Health Care Professional/Association - Other Health Care Professional
9	Akron Dermatology, Inc.	1	Unspecified	Health Care Professional/Association - Physician
10	Anacortes Dermatology Inc	1	Unspecified	Health Care Professional/Association - Physician
11	Andrew Paul Abraham PT PA DBA Therapy First	1	Unspecified	Physical Therapist - HC045
12	Anonymous	1	Unspecified	Unspecified
13	Atlanta Obstetrics and Gynecology Specialists	1	Unspecified	Unspecified
14	Birmingham Obstetrics Gynecology, P.C.	1	Unspecified	Health Care Professional/Association - Physician
15	Burlington ENT PC	1	Unspecified	Health Care Professional/Association - Physician
16	Caregiving Clarity Consulting	1	Unspecified	Unspecified
17	Clear Chronic Care	1	Unspecified	Health Care Professional/Association - Physician
18	Code Pause, Inc.	1	Unspecified	Unspecified
19	CSRA Renal Services	1	Unspecified	Health Care Professional/Association - Physician
20	Ear, Nose & Throat Associates of Worcester	1	Unspecified	Health Care Industry - PI015
21	Esyda Inc.	1	Unspecified	Health Care Industry - PI015
22	FirstVitals	1	Unspecified	Other Health Care Professional - HC075
23	Flannagan Plastic Surgery	1	Unspecified	Individual
24	HealthAdvanta	1	Unspecified	Health Care Professional/Association - Physician
25	HealthTech Solutions	1	Unspecified	Unspecified
26	Helios Integrated Medicine	1	Unspecified	Health Care Industry - PI015
27	Henderson Neurology	1	Unspecified	Physician - HC005
28	Hermitage Orthopedics and Sports Medicine PC	1	Unspecified	Other Health Care Provider - HPA70
29	Houston Dermatology and Skin Surgery, PLLC	1	Unspecified	Health Care Professional/Association - Physician
30	Illinois Rural Community Care Organization LLC	1	Unspecified	Other - OT001
31	Intellimedicine, PA	1	Unspecified	Health Care Industry - PI015
32	InVite Fitness	1	Unspecified	Consumer Group
33	It Is Well PLLC	1	Unspecified	Health Care Professional/Association - Nurse
34	Jacksonville Orthopaedic Institute	1	Unspecified	Health Care Professional/Association - Physician
35	Janning ENT Center, LLC	1	Unspecified	Health Care Provider/Association - Rural Health Clinic
36	K Dermatology	1	Unspecified	Government - Federal
37	Kidfixers Pediatrics	1	Unspecified	Health Care Professional/Association - Physician
38	lambe Industries	1	Unspecified	Health Care Industry - PI015
39	Lilac Ob-Gyn	1	Unspecified	Private Industry - Health Care
40	Little Fin Therapies	1	Unspecified	Speech-Language Therapist - HC055
41	Luis Reynoso, MD	1	Unspecified	Unspecified
42	Massive Bio, Inc	1	Unspecified	Private Industry - Health Care
43	MedOne Healthcare Partners	1	Unspecified	Health Care Provider/Association - Long-term Care
44	MiVision	1	Unspecified	Health Care Professional or Association - HC001
45	ModMed	1	Unspecified	Unspecified
46	national spine & pain centers	1	Unspecified	Health Care Professional/Association - Physician
47	National Spine and Pain Centers	1	Unspecified	Health Care Professional/Association - Physician
48	New Mexico	1	Unspecified	Health Care Professional/Association - Physician
49	Northwest eyelid and orbital specialist	1	Unspecified	Health Care Professional/Association - Physician
50	Ob Hospitalist Group	1	Unspecified	Health Care Professional/Association - Physician
51	Olympic Dermatology & Laser clinic	1	Unspecified	Health Care Professional/Association - Physician
52	Optimal Wellness Pharmacy	1	Unspecified	Government - Federal
53	OrthoCincy Orthopaedics & Sports Medicine, P.S.C.	1	Unspecified	Private Industry - Health Care
54	OSMS	1	Unspecified	Health Care Industry - PI015
55	Our Daughters Foundation	1	Unspecified	Health Care Industry - PI015
56	Peak Skin Center	1	Unspecified	Health Care Professional/Association - Physician
57	PhyxUp Health	1	Unspecified	Private Industry - Health Care
58	Portland Allergy and Asthma	1	Unspecified	Health Care Industry - PI015

Rank	Organization	Records	State(s)	Category/categories
59	PreciseDx	1	Unspecified	Device Industry - PI005
60	Precision Pain Care	1	Unspecified	Health Care Professional/Association - Physician
61	ProteusDx by FirstVitals	1	Unspecified	Device Industry - PI005
62	Puyallup Dermatology	1	Unspecified	Health Care Industry - PI015
63	Rayner Surgical Group	1	Unspecified	Drug Industry - PI010
64	Remo Health	1	Unspecified	Health Care Provider/Association - Other
65	Roswell OB/GYN	1	Unspecified	Unspecified
66	rtNOW, LLC	1	Unspecified	Health Care Provider/Association - Other
67	Schlessinger MD	1	Unspecified	Government - Federal
68	Sinus & Snoring Specialists	1	Unspecified	Health Care Professional/Association - Physician
69	Skin Specialists of the Capital Region	1	Unspecified	Health Care Professional/Association - Physician
70	Southern OB/GYN	1	Unspecified	Health Care Professional/Association - Physician
71	Speakeasy Therapy Services LLC	1	Unspecified	Health Care Professional/Association - Speech-Language Therapist
72	Spokane Valley Ear Nose & Throat PS	1	Unspecified	Private Industry - Health Care
73	St. Cloud Ear, Nose & Throat	1	Unspecified	Health Care Professional/Association - Physician
74	Sunrise Mentality LLC	1	Unspecified	Other - OT001
75	SurgicalRx	1	Unspecified	Health Care Industry - PI015
76	The Caring Board NFP	1	Unspecified	Individual
77	The Hirsh Center for Arthritis and Sports Medicine	1	Unspecified	Health Care Professional/Association - Physician
78	Trinity Health West Michigan	1	Unspecified	Other - Academic
79	TXd	1	Unspecified	Unspecified
80	Upstream rehab	1	Unspecified	Unspecified
81	Valar Labs	1	Unspecified	Device Industry - PI005
82	Virginia Beach Obstetrics & Gynecology	1	Unspecified	Health Care Professional/Association - Other Health Care Professional
83	Walter Reed NMMC	1	Unspecified	Academic - OT005
84	Willamette Valley Dermatology	1	Unspecified	Individual
85	Women's Healthcare Associates	1	Unspecified	Unspecified
86	Xandar Kardian	1	Unspecified	Device Industry - PI005

RPM/RTM policy-specific comment map

All 82 direct employee-only comments; named and withheld records

Comment ID	Submitter	State	Stance	Words
CMS-2026-2377-0079	CSRA Renal Services	Unspecified	Oppose/narrow	602
CMS-2026-2377-0131	Code Pause, Inc.	Unspecified	Oppose/narrow	3336
CMS-2026-2377-0165	Rick Pescatore	PA	Support/clarify	1010
CMS-2026-2377-0386	Brandy Kennedy	TX	Oppose/narrow	611
CMS-2026-2377-0407	rtNOW, LLC	Unspecified	Oppose/narrow	447
CMS-2026-2377-0443	Anonymous/withheld	Unspecified	Oppose/narrow	17763
CMS-2026-2377-0590	Xandar Kardian	Unspecified	Oppose/narrow	1675
CMS-2026-2377-0614	Lance Martin	TX	Oppose/narrow	292
CMS-2026-2377-0655	Anonymous/withheld	Unspecified	Oppose/narrow	687
CMS-2026-2377-0694	Sara Spencer-Kinoti	TX	Support/clarify	446
CMS-2026-2377-0770	Massive Bio, Inc	Unspecified	Oppose/narrow	4899
CMS-2026-2377-0797	SurgicalRx	Unspecified	Oppose/narrow	1019
CMS-2026-2377-0857	PhyxUp Health	Unspecified	Oppose/narrow	1949
CMS-2026-2377-0960	Precision Pain Care	Unspecified	Oppose/narrow	451
CMS-2026-2377-0964	Christine Blasiolo	Unspecified	Oppose/narrow	660
CMS-2026-2377-1013	Esyda Inc.	Unspecified	Oppose/narrow	589
CMS-2026-2377-1024	Devon Gaur	CA	Oppose/narrow	3340
CMS-2026-2377-1040	Anonymous/withheld	Unspecified	Oppose/narrow	176
CMS-2026-2377-1078	InVite Fitness	Unspecified	Oppose/narrow	574
CMS-2026-2377-1081	Andrea Lausch	Unspecified	Oppose/narrow	660
CMS-2026-2377-1094	Anonymous/withheld	Unspecified	Oppose/narrow	356
CMS-2026-2377-1114	R Scott Harris	Unspecified	Oppose/narrow	259
CMS-2026-2377-1116	Meredith Pieper	Unspecified	Oppose/narrow	660
CMS-2026-2377-1121	Lori Pearlmutter	Unspecified	Oppose/narrow	660
CMS-2026-2377-1145	Gary Lin	Unspecified	Oppose/narrow	660
CMS-2026-2377-1148	ProteusDx by FirstVitals	Unspecified	Oppose/narrow	1872
CMS-2026-2377-1157	Daniel Rothbaum	FL	Oppose/narrow	272
CMS-2026-2377-1162	Shannon Forrestall	Unspecified	Oppose/narrow	660
CMS-2026-2377-1171	Michael Marzano	Unspecified	Oppose/narrow	660
CMS-2026-2377-1172	Tammy Richmond	Unspecified	Oppose/narrow	254

Comment ID	Submitter	State	Stance	Words
CMS-2026-2377-1190	Meghan Mosher	Unspecified	Oppose/narrow	254
CMS-2026-2377-1198	Tamara Bazar	Unspecified	Oppose/narrow	254
CMS-2026-2377-1231	Jon-Michial Carter	Unspecified	Oppose/narrow	254
CMS-2026-2377-1233	Illinois Rural Community Care Organization LLC	Unspecified	Oppose/narrow	3117
CMS-2026-2377-1235	Ron Lobato	Unspecified	Oppose/narrow	254
CMS-2026-2377-1236	Renee Halfhill	Unspecified	Oppose/narrow	660
CMS-2026-2377-1243	Saad Junaid	Unspecified	Oppose/narrow	254
CMS-2026-2377-1247	Daniel Macera	Unspecified	Oppose/narrow	660
CMS-2026-2377-1265	Krisi Probert	GA	Oppose/narrow	1937
CMS-2026-2377-1266	Katie Abebe	PA	Oppose/narrow	465
CMS-2026-2377-1269	Kyle Warner	Unspecified	Oppose/narrow	660
CMS-2026-2377-1287	Branden Bubnick	KY	Oppose/narrow	479
CMS-2026-2377-1288	Jamie Roberts	TN	Oppose/narrow	494
CMS-2026-2377-1292	Matthew O'Connor	TN	Oppose/narrow	448
CMS-2026-2377-1293	Matthew Johnson	GA	Oppose/narrow	521
CMS-2026-2377-1304	Andreanna McLeod	MO	Oppose/narrow	448
CMS-2026-2377-1306	Jennifer Cazeaux	GA	Oppose/narrow	517
CMS-2026-2377-1307	Meagan Paczesny	MS	Oppose/narrow	446
CMS-2026-2377-1308	Luis Reynoso, MD	Unspecified	Oppose/narrow	45
CMS-2026-2377-1313	Mark Jagger	MO	Oppose/narrow	457
CMS-2026-2377-1319	Joshua Crumley	Unspecified	Oppose/narrow	454
CMS-2026-2377-1332	Grant Stophlet	Unspecified	Oppose/narrow	450
CMS-2026-2377-1336	Sarah Alcorn	PA	Oppose/narrow	690
CMS-2026-2377-1339	Greg Schroeder	UT	Oppose/narrow	416
CMS-2026-2377-1349	Mason Williams	GA	Oppose/narrow	322
CMS-2026-2377-1352	Nathan Koslakiewicz	KY	Oppose/narrow	474
CMS-2026-2377-1363	Jodhvir Sarai	WA	Support/clarify	365
CMS-2026-2377-1370	Alicia Graziano	DE	Oppose/narrow	468
CMS-2026-2377-1372	Kleun McCrimon	AL	Oppose/narrow	484
CMS-2026-2377-1384	Stephen Weyandt	GA	Oppose/narrow	411
CMS-2026-2377-1386	Annie Reybrock	Unspecified	Oppose/narrow	660
CMS-2026-2377-1388	Eric Steed	GA	Oppose/narrow	497
CMS-2026-2377-1390	Jameson Riner	OH	Oppose/narrow	439
CMS-2026-2377-1395	Kathleen Morse	Unspecified	Oppose/narrow	489
CMS-2026-2377-1397	Upstream rehab	Unspecified	Oppose/narrow	368
CMS-2026-2377-1412	Mitch Hutter	Unspecified	Oppose/narrow	660
CMS-2026-2377-1420	Richard Ball	Unspecified	Oppose/narrow	660
CMS-2026-2377-1430	Andrew Wallner	Unspecified	Oppose/narrow	660
CMS-2026-2377-1433	Nicole Wentzien	Unspecified	Oppose/narrow	660
CMS-2026-2377-1438	Keith Haviland	Unspecified	Oppose/narrow	660
CMS-2026-2377-1445	Brandon Anible	OR	Oppose/narrow	711
CMS-2026-2377-1446	Optimal Wellness Pharmacy	Unspecified	Oppose/narrow	480
CMS-2026-2377-1449	Christine Yoder	PA	Oppose/narrow	750
CMS-2026-2377-1486	Lisa Williams	Unspecified	Oppose/narrow	660
CMS-2026-2377-1532	Parag Mehta	NY	Oppose/narrow	1033
CMS-2026-2377-1533	Jeff Smietana	Unspecified	Oppose/narrow	562
CMS-2026-2377-1599	Jason Taylor	TN	Oppose/narrow	445
CMS-2026-2377-1619	Matthew Foley	GA	Oppose/narrow	446
CMS-2026-2377-1632	Harold Rahnner	Unspecified	Oppose/narrow	487
CMS-2026-2377-1633	David Gustafson	NC	Oppose/narrow	498
CMS-2026-2377-1634	Sarah Roberts	Unspecified	Oppose/narrow	254
CMS-2026-2377-1641	Chelsea Horgan	NC	Oppose/narrow	471

Theme codebook

Operational definitions used for deterministic multi-label coding

Theme	Operational definition
Payment adequacy, conversion factor & practice expense	Payment rates, conversion factor, practice expense, RVUs, efficiency adjustments, or reimbursement reductions.
Access, rural burden & consolidation	Patient access, rural or underserved burden, wait/travel effects, workforce constraints, independent-practice viability, or consolidation.
Same-day E/M, modifier 25 & global procedures	Same-day evaluation/management, modifier 25, separately identifiable services, global periods, or related payment reductions.

Theme	Operational definition
Specialty and service valuation	Specialty-specific codes, service valuation, or impacts on named clinical disciplines.
Remote physiologic/therapeutic monitoring	RPM, RTM, remote patient monitoring, associated codes, staffing, device supply, or operational requirements.
Chronic care, coaching & navigation	Health or nurse coaching, chronic care, advance care planning, CHI, PIN, or navigation services.
Telehealth and virtual access	Telehealth, telemedicine, virtual/audio-only care, FQHCs, RHCs, or remote access.
Laboratory, diagnostics & interoperability	Laboratory or diagnostic policy, duplicate testing, molecular/genomic testing, pathology, or interoperability.
AI, software and human oversight	Artificial intelligence, software-mediated clinical work, algorithms, clinical decision support, or human oversight.
Drug payment, pharmacy & 340B	Prescription drug payment, pharmacy services, rebates, Part D, or 340B.
Behavioral health and substance use	Mental or behavioral health, substance use, addiction, psychiatry, psychology, or opioid policy.
Wound care & skin substitutes	Wound management, skin substitutes, or debridement.
Value-based care, ACOs & MSSP	ACOs, MSSP, shared savings, QPP/MIPS, or value-based payment.
Prevention, vaccines & screening	Prevention, vaccines, immunization, screening, or annual wellness.
Medicare eligibility & immigration	Eligibility rules involving immigration or lawful-presence categories.

Coding limitations

- Theme flags use transparent keyword rules over combined inline and attachment text. They improve consistency and auditability but can produce false positives and false negatives.
- A comment may receive multiple themes, so percentages do not sum to 100%.
- Theme prevalence measures mentions, not support or opposition. Only the employee-only RPM/RTM subset received complete manual stance review.
- Attachment OCR may contain recognition errors, especially for low-resolution scans, unusual layouts, signatures, or handwriting.
- The codebook is suitable for corpus navigation and descriptive analysis; it is not a substitute for legal review of a specific comment.

Methods, reproducibility, and limitations

What was collected, how it was analyzed, and what remains uncertain

Collection

The analysis retrieved every public comment detail record reachable for docket document CMS-2026-2377-0002 at the snapshot time. Each detail request included attachment metadata. One preferred representation was downloaded for each of 113 logical attachments; machine-readable text was extracted where available, and image-only pages were OCR-processed.

Identity and ranking

The public organization field takes precedence when present; otherwise the public first and last name fields are combined. Blank or anonymous values are classified as anonymous/withheld. Exact displayed names are grouped, but legal entities, aliases, abbreviations, and spelling variants are not merged without independent verification.

Duplicate detection

Text is normalized and compared using exact hashes and five-word shingles with a conservative near-duplicate threshold. Short attachment cover notes are excluded from duplicate treatment because their underlying documents may differ. The resulting unique-language count is heuristic and should be reported as approximate.

Snapshot limitations

- The docket remained open at collection; later submissions will change counts and may change theme and stance distributions.
- Only public fields and public attachments are analyzed. Withheld identities, non-public material, agency moderation decisions, and later modifications are outside scope.
- Self-reported category, geography, name, and organization fields are incomplete and not independently verified.
- Counts describe the record; they do not establish representativeness of Medicare beneficiaries, clinicians, practices, vendors, or the general public.
- Client recommendations are strategic interpretations of the record, not legal advice or a prediction of CMS's final rule.

Audit files in the companion data package

- All 1,595 coded comment rows and public metadata.
- Complete named submitter census and explicit top-100 extract.
- All 99 RPM/RTM-related records and the 82 policy-specific stance decisions.
- All 75 duplicate clusters and their member comment IDs.

- Daily volume, state, category, submitter-type, length, theme-overlap, codebook, organization, and RPM subtheme exhibits.

Sources

Regulations.gov public comment record: <https://www.regulations.gov/document/CMS-2026-2377-0002/comment>

Regulations.gov API documentation: <https://open.gsa.gov/api/regulationsgov/>

CMS CY 2027 Physician Fee Schedule proposed-rule materials:

<https://www.cms.gov/medicare/payment/fee-schedules/physician/federal-regulation-notice/cms-1848-p>

Client context: CareTrack-RPM-Advocacy-Proposal.md, provided August 3, 2026.